

Riverside Medical Center — Department of Cardiology

INPATIENT DISCHARGE SUMMARY

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY — CONTAINS NO REAL PATIENT DATA

Patient: Robert T. Harmon	DOB: 03/14/1966	MRN: 4821093
Date: November 8, 2024	Provider: Dr. Vivian Park, MD — Interventional Cardiology	

ADMISSION DIAGNOSES

Primary Dx:	Acute ST-Elevation Myocardial Infarction (STEMI), anterior wall (ICD-10: I21.09)	Secondary Dx:	Hypertension (I10), Hyperlipidemia (E78.5), Former tobacco use (Z87.891)
Admitted:	November 4, 2024 via Emergency Department	Discharged:	November 8, 2024 (4-day stay)
Disposition:	Home with outpatient cardiology follow-up		

HOSPITAL COURSE

Mr. Harmon, a 58-year-old male with known hypertension and hyperlipidemia, presented to the Emergency Department with acute onset substernal chest pressure radiating to the left arm, associated with diaphoresis and shortness of breath. ECG on arrival demonstrated 3mm ST elevations in leads V1-V4 consistent with anterior STEMI. He was taken emergently to the cardiac catheterization laboratory.

Coronary angiography revealed 100% thrombotic occlusion of the proximal left anterior descending (LAD) artery. Successful primary percutaneous coronary intervention (PCI) was performed with aspiration thrombectomy followed by deployment of a 3.5 x 28mm Synergy drug-eluting stent to the proximal LAD. TIMI III flow was restored. Residual disease in the mid-circumflex (60% stenosis) was noted and deferred for staged intervention.

Post-procedure, the patient was monitored in the Cardiac Care Unit for 48 hours. Echocardiogram on hospital day 2 demonstrated an ejection fraction of 42% with anterior wall hypokinesis. He remained hemodynamically stable throughout, with no arrhythmias, recurrent ischemia, or heart failure. He was advanced to a cardiac rehabilitation-appropriate activity level prior to discharge.

DISCHARGE MEDICATIONS

Aspirin:	81mg PO daily — lifelong (do not discontinue)	Ticagrelor (Brilinta):	90mg PO twice daily x 12 months minimum
Metoprolol Succinate:	25mg PO daily — titrate at follow-up	Atorvastatin:	80mg PO nightly — high-intensity statin
Lisinopril:	5mg PO daily — for reduced EF post-MI	Nitroglycerin SL:	0.4mg PRN chest pain — call 911 if no relief in 5 min

FOLLOW-UP INSTRUCTIONS

Cardiology follow-up is scheduled with Dr. Park in 7 days (November 15, 2024) at Riverside Heart Institute. A repeat echocardiogram will be performed at 6 weeks to reassess ejection fraction and guide further titration of heart failure medications.

Activity restrictions: No lifting over 10 pounds for 2 weeks. Resume walking gradually as tolerated. No driving for 1 week or while taking opioid pain medications. Sexual activity may resume after clearance at follow-up visit.

Diet: Low sodium (<2g/day), low saturated fat. Cardiology dietitian referral placed. Cardiac rehabilitation referral submitted to outpatient CR program — patient should expect contact within 2 weeks.

Emergency precautions: Return to ED immediately for any recurrent chest pain, shortness of breath at rest, palpitations, lightheadedness, or leg swelling.

This document contains entirely synthetic, AI-generated data for demonstration purposes only. No real patient, provider, or institutional information is represented.